GARHWAL HISTOPATHOLOGY & CYTOLOGY CENTRE
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Lab Reg. No. UK/DDN/PL/2012/0147

SURGICAL PATHOLOGY REPORT

Patient Name:	**Ms. NANDITA RAWAT**	Accession No.:	**GHC/S/26/1027**
Age / Sex:	34 Yrs / F	Received:	04-03-2026
Clinician:	Dr. Manish Uniyal, Rispana Surgical & Endocrine Centre	Reported:	**11-03-2026**
Specimen:	Total thyroidectomy with bilateral level VI nodes (operated 03-03-2026)	Previous:	FNAC GHC/C/26/0211

Gross

Total thyroidectomy specimen weighing 24 g; right lobe 5 x 3 x 2.5 cm, left lobe 4.5 x 2 x 1.8 cm, isthmus 1.5 cm. A grey-white firm unencapsulated nodule 2.6 x 2.2 x 2 cm in the right lobe with gritty cut surface, reaching close to the posterior capsule. Left lobe unremarkable. Right level VI: 6 nodes; left level VI: 5 nodes.

Microscopy

Papillary thyroid carcinoma, classic type: complex branching papillae lined by cells with overlapping, optically clear nuclei, nuclear grooves and pseudo-inclusions; psammoma bodies present. Focal extension into perithyroidal fat (microscopic). Background lymphocytic thyroiditis.

Synoptic summary

Parameter	Finding
Histological type	Papillary thyroid carcinoma, classic (conventional) type
Focality / size	Unifocal; 2.6 cm, right lobe
Extrathyroidal extension	Microscopic only (perithyroidal soft tissue); no gross extension
Lymphovascular invasion	Present, focal (fewer than 4 vessels)
Margin status	Uninvolved by carcinoma; closest 0.8 mm (posterior capsule, right lobe)
Lymph nodes	Right level VI: 4/6 involved (largest deposit 0.9 cm, no extranodal extension); left level VI: 0/5
Parathyroid	One parathyroid gland in the specimen (incidental)
pTNM (AJCC 8th ed.)	**pT2 pN1a**

Diagnosis

Total thyroidectomy: PAPILLARY THYROID CARCINOMA, classic type, 2.6 cm, right lobe; metastatic carcinoma in 4 of 11 central compartment lymph nodes.

Dr. Ritu Bisht
M.D. (Pathology)
Reg. No. UKMC-3051
Consultant Pathologist

*** End of Report ***

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY